

(1) 32YOM Lightning strike secondary to Horseback Riding Accident

Scenario Set Up	Equipment: SAM Splint, ace wrap/ roller gauze
Dispatch	Respond C3 for an unknown medical
Scene Size Up	32YOM was taking his horse down the old town road - riding on an empty horse trail and was Struck by lightning on the shoulder. Strike caused the <i>horse</i> to freak out and buck off the rider. Pt is found on the back shouting in pain. - Patient rode till he cant no more Attention to proctors: Pt is deaf due to lighting strike. Patient can't hear at all. Can talk but will only understand questions by using alternate modes of communication such as writing questions on a paper/ phone. Pt can only yell as they cant hear themselves.
Pertinent Primary Assessment Findings	AVPU - A&Ox3, unknown as to what happened. Skin signs - Warm and dry Airway - clear Breathing - 24RPM fast but not labored Circulation - Strong, Rapid, Irregular radial pulses, Cap Refill 1s
Pertinent Secondary Assessment Findings	Events: Pt was horseback riding when suddenly struck by lightning. Lost consciousness and Woke up with horse gone and lying on back. Pt is dizzy and a bit disorientated. Unable to hear due to eardrums being ruptured. Can only understand through written communication. When speaking can only yell. Patient is complaining of burning pain in stomach. Assessment findings: - Superficial electrical burn / entry point on Left shoulder. (or wherever the entry point moulage is) - Superficial electrical burn/ exit point on lower left leg (or wherever the exit point moulage is) - Tenderness in abdomen - Keraunoparalysis of the lower left leg(A neurological phenomenon causing temporary paralysis of an extremity following lightning strike. Usually spontaneously resolves itself within a few hours) - Transverse radius fracture A - None M - Meditation, Lexapro P - Anxiety L - Breakfast E - <i>see above</i>

	 neighhhhh
Vitals	Initial: HR: 155, BP: 140/90, RR: 24, SPO2: 95%, BGL: 89
Treatments	Splint arm in position of function, wrap burns in dry gauze. Treat for shock!!! Provide blanket.
Key Points	- Know signs and symptoms of lightning strikes - It is important to find alternate forms of communication when a patient is unable to see/talk/hear. Just because they can't respond doesn't mean that the patient is unresponsive. Think for ways outside the box to communicate. Be creative! - Electrical burn / lightning strike is automatic transfer to a burn center in ALCO - Lightning can cause internal visceral injuries and bone fractures - Deafness, and loss of sight are common following lightning strikes. - No need to backboard as not indicated.

hORseE fAcTs 	• Horses can sleep both lying down and standing up. • Horses can run shortly after birth. • Domestic horses have a lifespan of around 25 years. • A 19th century horse named 'Old Billy' is said to have lived 62 years. • Horses have around 205 bones in their skeleton. • Horses have bigger eyes than any other mammal that lives on land. • Because horse's eyes are on the side of their head they are capable of seeing nearly 360 degrees at one time. • Estimates suggest that there are around 60 million horses in the world. • Scientists believe that horses have evolved over the past 50 million years from much smaller creatures.
--	--

(2) 44 YOM Hypothermia & 40 YOF CO

Scenario Set Up	Car, Backboard, O2, blanket
Dispatch	Respond C2 for a car stuck in the woods.
Scene Size Up	Car is stuck in a snowbank after sliding on ice. Husband left to call for help and lead emergency crew to stuck car. Wife stayed in car with the heater cranked all the way up while the husband went to get help. Very cold and windy outside. Husband is suffering from hypothermia and frostbite. Since the exhaust pipe is covered by snow, the car is filled with CO and the wife has CO poisoning.
Pertinent Primary Assessment Findings	44 YOM: cc “really cold” A&Ox4 A - Clear B - Slow and shallow C - Pulse slow and weak, skin pale cold dry, fingertips and nose cyanotic (frostbite) 40 YOF: cc “headache” A&Ox1 A - Clear B - Fast and labored C - Pulse fast and weak, skin red warm dry Wife BECOME UNRESPONSIVE after primary assessment. Husband FREAK OUT when she becomes unresponsive, i.e. run over and shake her body and yell at EMTs.
Pertinent Secondary Assessment Findings	44 YOM: S - Feels cold, shivering, hands and face hurt, speech slightly slurred, slightly confused A - None M - metformin, lisinopril P - prediabetic, HTN L - McDonald’s this morning E - Walked through the snow to get help If no explicit cold treatment is given, become more confused and eventually go unresponsive 40 YOF: S - Headache/confused -> unresponsive, A - Amoxicillin M - methadone(reduce withdrawal symptoms by targeting the same centers in the brain that opioids target) P - struggled with opioid addiction, currently in recovery L - McDonald’s this morning E - Sitting in car with the engine on, waiting for help

Vitals	40 YOF: HR 124, RR 24, SPO2 100%, Temp 98.6 44 YOM: HR 68, RR 12, SP02 86, Temp 95.5
Treatments	40 YOF: O2 15 lpm NRB 44 YOM: O2 optional, remove from cold, blanket. Rewarm extremities with blankets/warm objects
Key Points	Important to both remove patients from harmful environments and treat them
Bonus Questions	When would you not warm frozen extremities? (If they might refreeze) What will the SPO2 of someone with CO poisoning say? Why?